

COMPREHENSIVE INPATIENT PROGRESS NOTE

UNIVERSITY MEDICAL CENTER

Inpatient Hematology Service

Progress Note - Hospital Day 42

Date: March 15, 2024

Time: 08:30

Patient: Helen Perez

MRN: AML046

DOB: 05/08/1982

Admission Date: 02/02/2024

Room: 7 North, Bed 12

SUBJECTIVE: Ms. Perez is a 41-year-old previously healthy mother of three who is currently day +26 from 7+3 re-induction (second cycle) for favorable-risk AML with biallelic CEBPA mutations (bZIP in-frame mutations confirmed by NGS, VAF 48% and 44% respectively). She states, "I finally feel human again - the fevers have stopped and I actually ate a full breakfast." She reports resolution of her severe mucositis (previously grade 3 requiring PCA), improved energy, and decreasing bone pain. She's been able to FaceTime with her children daily which has significantly improved her mood. Denies chest pain, shortness of breath, or bleeding. Last fever was 72 hours ago.

OBJECTIVE:

Vital Signs:

T 36.8°C, HR 78, BP 118/72, RR 16, O2 98% RA, Weight 61.2 kg (up from nadir of 58.4 kg)

Physical Examination:

General: Alert, oriented, appears stated age, no acute distress. Notably improved affect compared to last week

HEENT: Mucositis improving (now grade 1), can see only minor ulceration on soft palate. No gingival hypertrophy or bleeding. No scleral icterus

Cardiovascular: Regular rate and rhythm, no murmurs/rubs/gallops. No lower extremity edema

Pulmonary: Clear to auscultation bilaterally, no wheezes/rales/rhonchi

Abdomen: Soft, non-tender, non-distended. Normoactive bowel sounds. No hepatosplenomegaly

Skin: Resolving drug rash on trunk (previously extensive maculopapular rash attributed to cefepime). Multiple bruises in various stages of healing

Neurologic: CN II-XII intact, strength 5/5 throughout, sensation intact

Lines: Right PICC line without erythema or drainage, draws and flushes well

Laboratory Data (03/15/24):

- **CBC:** WBC 1.8 (ANC 810 - first day >500!), Hgb 8.2 (s/p 1 unit pRBCs yesterday), Plt 42K (transfused this AM)
- **Chemistry:** Na 138, K 3.9, Cl 102, CO2 24, BUN 18, Cr 0.7 (baseline 0.8), Glucose 92

- **Hepatic:** ALT 45 (down from peak 156), AST 38, Alk Phos 102, Total Bili 0.9, Albumin 3.1
- **Coagulation:** PT 12.1, INR 1.0, PTT 28
- **Infectious:** Blood cultures 03/12, 03/13, 03/14 - all no growth to date
- **Inflammatory markers:** CRP 12 (down from 89), Ferritin 1,843 (down from 4,200)

Day 14 Bone Marrow Biopsy (03/01/24): Hypocellular marrow (<5% cellularity) with no morphologic evidence of leukemia. Flow cytometry: No abnormal myeloid blast population detected. Awaiting cytogenetics and CEBPA mutation analysis.

ASSESSMENT:

This is a 41-year-old woman with favorable-risk AML (biallelic CEBPA mutations) who is day +26 from cycle 2 of 7+3, now with count recovery beginning (ANC >500 for first time today). Her initial presentation on 10/12/23 included WBC 67K with 72% circulating blasts, profound thrombocytopenia, and DIC which has all resolved. First induction cycle (10/19/23) resulted in CR with incomplete count recovery (CRi). She remained in remission for 4 months before starting consolidation.

Key Issues:

1. **AML with biallelic CEBPA mutations - Favorable risk**
 - Completed re-induction due to residual disease on day 14 marrow after cycle 1
 - Day 14 marrow from current cycle shows aplasia without leukemia
 - Count recovery starting (ANC 810 today)
 - Plan for 3-4 cycles of HiDAC consolidation once counts recover
 - Excellent prognosis with 5-year OS ~60-70% for biallelic CEBPA
2. **Prolonged neutropenia - Recovering**
 - 31 days of severe neutropenia (ANC <100)
 - Complicated by multiple febrile episodes, now resolved
 - G-CSF started day +21, continuing
3. **Febrile neutropenia - Resolved**
 - Multiple organisms: Strep viridans bacteremia (day +8), C. diff (day +15), possible fungal PNA (day +18)
 - Completed 14 days meropenem, 10 days PO vancomycin
 - Currently on voriconazole for presumed fungal coverage
 - Afebrile x72 hours
4. **Nutritional status - Improving**
 - Severe mucositis led to 3kg weight loss
 - Now tolerating full PO diet
 - TPN discontinued 2 days ago
 - Nutrition following daily
5. **Psychosocial**
 - Significant distress about prolonged separation from children (ages 6, 9, 12)
 - Psychology consulted, started on escitalopram 10mg daily
 - Husband very supportive, at bedside daily
 - Social work assisting with FMLA paperwork for husband

PLAN:**1. Hematologic:**

- Continue G-CSF 480mcg SubQ daily until ANC >1000 x2 days
- Transfuse pRBCs for Hgb <7, platelets for <10K or bleeding
- Repeat bone marrow once ANC >1000 to confirm CR
- Plan discharge once ANC >1000 and afebrile

2. Infectious Disease:

- Continue voriconazole 200mg PO BID through neutropenia
- Continue acyclovir and prophylactic levofloxacin
- May discontinue once ANC consistently >1000

3. Supportive Care:

- Continue PPI for GI prophylaxis
- Mouth care QID with magic mouthwash
- Daily weights, strict I&Os
- Fall precautions given thrombocytopenia

4. Discharge Planning:

- Anticipated discharge in 3-5 days if counts continue to recover
- Will need twice weekly labs as outpatient
- Follow-up in clinic within 48 hours of discharge
- Consolidation cycle #1 tentatively planned for 2 weeks post-discharge

CONSULTANTS FOLLOWING:

- Infectious Disease (Dr. Martinez)
- Nutrition (Following)
- Psychology (Dr. Smith)
- Social Work (Jennifer RN)

Electronically signed by:
Michael Chen, MD
Attending Hematologist
Pager: 4567

Addendum 10:30: Cytogenetics from day 14 marrow just resulted - normal female karyotype 46,XX. CEBPA mutations still pending.